EXECUTIVE SUMMARY

Target Audience: American Hospital Association & Centre for Medicaid and Medicare Services

The Problem:

Systemic Failures: Cases like Farid Fata (fraudulent chemo) and Martin Shkreli (5,000% drug markup).

Barriers: Dismissive providers, unstable funding, lack of tech integration.

The Impact of Advocacy : Advocates reduce disparities, improve outcomes, and lower costs by addressing social determinants of health (SDOH).

Recommendations

1. Combat Provider Dismissive attitude
2. Mandating Advocates as Standard of Care
3. Leverage Technology
4. Diversify Funding

AGENDA

01 Introduction, History and Background

02 Patient Journey Illustrations

03 Barriers and Recommendations

04 Conclusion and References

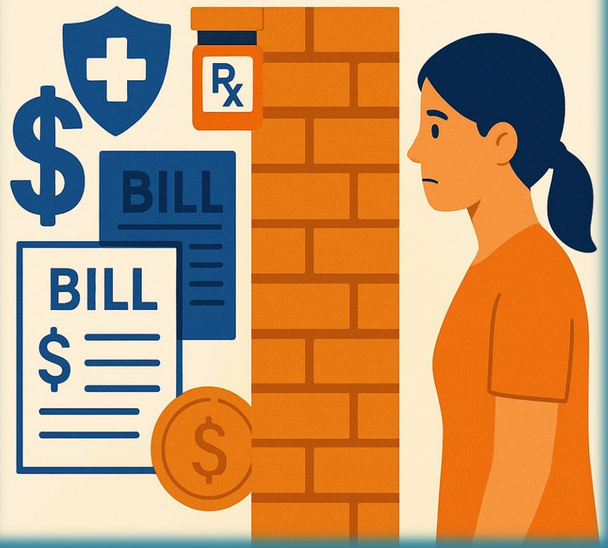
Healthcare Disparities

UNEQUAL ACCESS TO HEALTHCARE

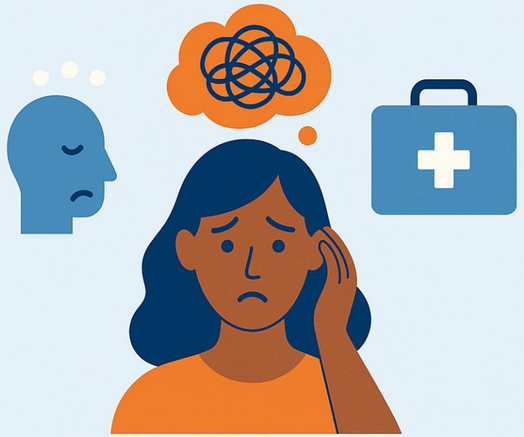
- Geographic barriers (rural vs. urban healthcare availability)
- Lack of healthcare facilities in underserved areas
- Long wait times and limited provider availability

FINANCIAL BARRIERS

- High cost of medical care and insurance coverage gaps.
- Lack of affordable prescription medications.
- Medical debt and its impact on patients' well-being.



MENTAL HEALTH DISPARITIES



- Stigma surrounding mental health in certain communities
- Limited access to mental health professionals and resources
- Insurance coverage gaps for mental health services

Racial & Ethnic Disparities

- Disproportionate health outcomes in minority populations.
- Implicit bias in medical decision-making.
- Language and cultural barriers affecting care

Who are patient advocates ?

PATIENT ADVOCATES

Individuals who support and guide patients through the healthcare system

What Do Patient Advocates Do?

Guiding Patients Through Healthcare Processes

Help patients understand diagnoses, treatment options, and medical procedures

Ensuring Patients' Rights

Educate patients about their rights regarding informed consent, medical records, and treatment decisions

Assisting with Insurance and Billing

Help patients understand medical bills and insurance claims

Advocating for Special Populations

Work with elderly, disabled, or chronically ill patients who need additional assistance. Ensure access to necessary healthcare services for underserved populations

Providing Emotional Support

Offer counseling and emotional support to patients and families dealing with serious illnesses

Timeline

1972

Society for Healthcare Consumer Advocacy (SHCA

Established under the American Hospital Association (AHA), this was one of the first formal groups to train and organize patient representatives

1993

Case Management Society of America (CMSA)

Emerged as a professional organization for case managers, many of whom incorporated patient advocacy into their roles, blending clinical and advocacy skills.

2003

National Association of Healthcare Advocacy Consultants (NAHAC)

Founded to unify independent patient advocates and create standards for the emerging private advocacy field.

2006

Patient Advocate Foundation (PAF)

Expanded its role in providing direct advocacy services (e.g., insurance navigation, financial aid), legitimizing advocacy as a critical support system for patients.

2009

Professional Patient Advocate Certification

The Patient Advocate Certification Board (PACB) launched the Board Certified Patient Advocate (BCPA) credential, the first nationally recognized certification for patient advocates.

2016

Oregon Licenses Healthcare Advocates

Oregon became the first state to require licensing for private patient advocates, setting a precedent for state-level regulation of the profession.

2019

National Alliance of Healthcare Patient Advocates (NAHPA)

Formed to promote ethical standards and collaboration among advocacy professionals, reflecting the field's growing maturity.

Two Mothers, Two Outcomes: The Power of Advocacy in Maternal Healthcare

How a Patient Advocate Transformed Maria's Healthcare Journey?

The advocate ensured Maria had access to interpreters, bridging the language barrier and enabling her to understand her diagnosis, treatment options, and medical instructions accurately.

By facilitating communication among various healthcare providers, the advocate streamlined Maria's care, preventing missed appointments and ensuring that all specialists were aligned in her treatment plan.

The advocate helped Maria navigate the complexities of the healthcare system, addressing insurance denials, appealing claims, and connecting her to available financial resources.

Through clear explanations and support, the advocate empowered Maria to make informed decisions about her care, follow her treatment plan, and confidently advocate for her own health needs.

The Ripple Effect of Healthcare Advocacy

Without Advocate

Missed Appointments

↓

Misunderstood Treatments

↓

Poor Outcomes

With Advocate

Timely Care

↓

Clear Communication

↓

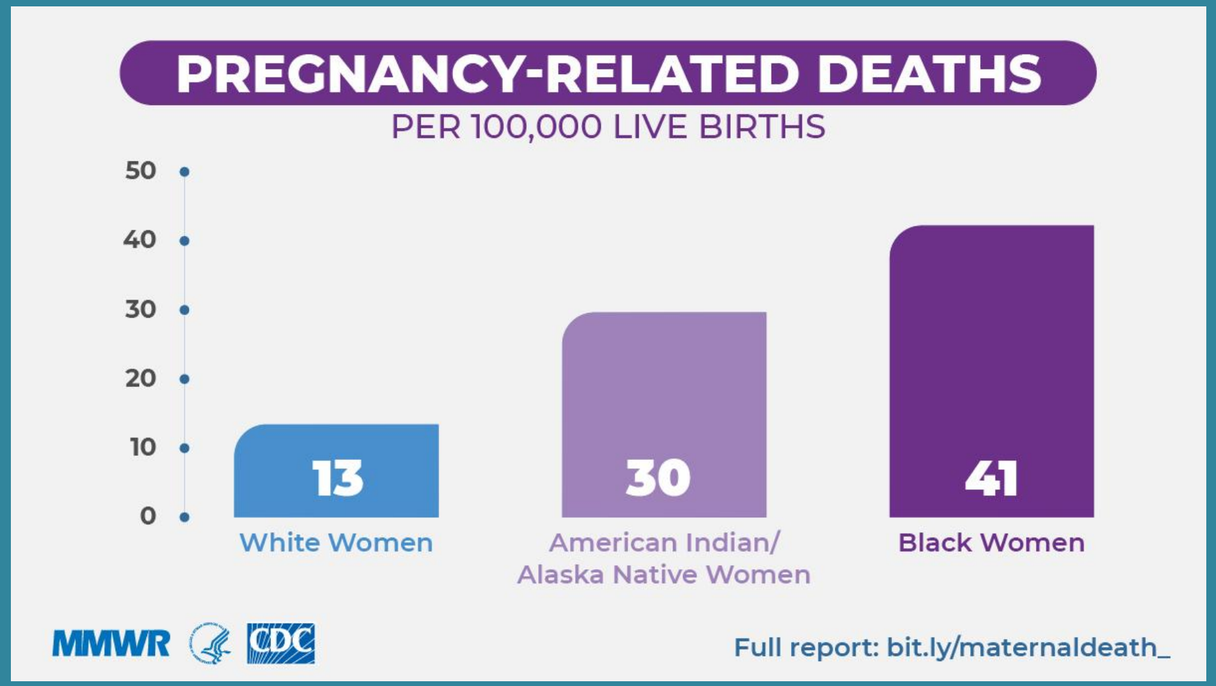
Adherence

↓

Recovery

49.5
per 100,000
Non-Hispanic Black or African American mothers

13.2
per 100,000
Non-Hispanic Asian mothers



Profiting from Pain: The Crimes of Martin Shkreli and Farid Fata

- Farid Fata was a Michigan-based oncologist who committed one of the most egregious medical frauds in U.S. history. He intentionally misdiagnosed healthy patients with cancer and administered unnecessary chemotherapy, radiation, and other toxic treatments—all to fraudulently bill Medicare and private insurers.

- Martin Shkreli, often called the "Pharma Bro," gained notoriety for dramatically increasing the price of Daraprim, a life-saving drug used to treat parasitic infections in AIDS and cancer patients. In 2015, as CEO of Turing Pharmaceuticals, he raised the price from 13.50 to 750 per pill—a 5,000% hike.

Societal Barrier: Providers dismiss advocates as “not essential”.

Recommendation: Combat Provider Dismissive Attitude

01

Why It Matters?

This bias stems from a lack of understanding of advocates’ roles in addressing social determinants of health (SDOH), cultural barriers, and care coordination

02

How does it impact?

Advocates build trust by representing patient voices and explaining care in culturally relevant ways.
Advocates bridge gaps by coordinating follow-up care, translating instructions, and ensuring medication access.

03

How to implement?

Mandate Advocate-Provider Co-Training, where clinicians witness firsthand how advocates resolve SDOH barriers. Policy Reforms which Legally require advocate inclusion in care plans to qualify for accreditation

Policy Barrier: No Medicare/Medicaid reimbursement for advocate services.

Recommendation: Prescribing Partnerships: Mandating Advocates as Standard of Care

01
Why It Matters?

Value-based care models aim to prioritize outcomes over volume, and this policy ensures providers address root causes of disparities to earn reimbursements, aligning financial incentives with equitable care delivery.

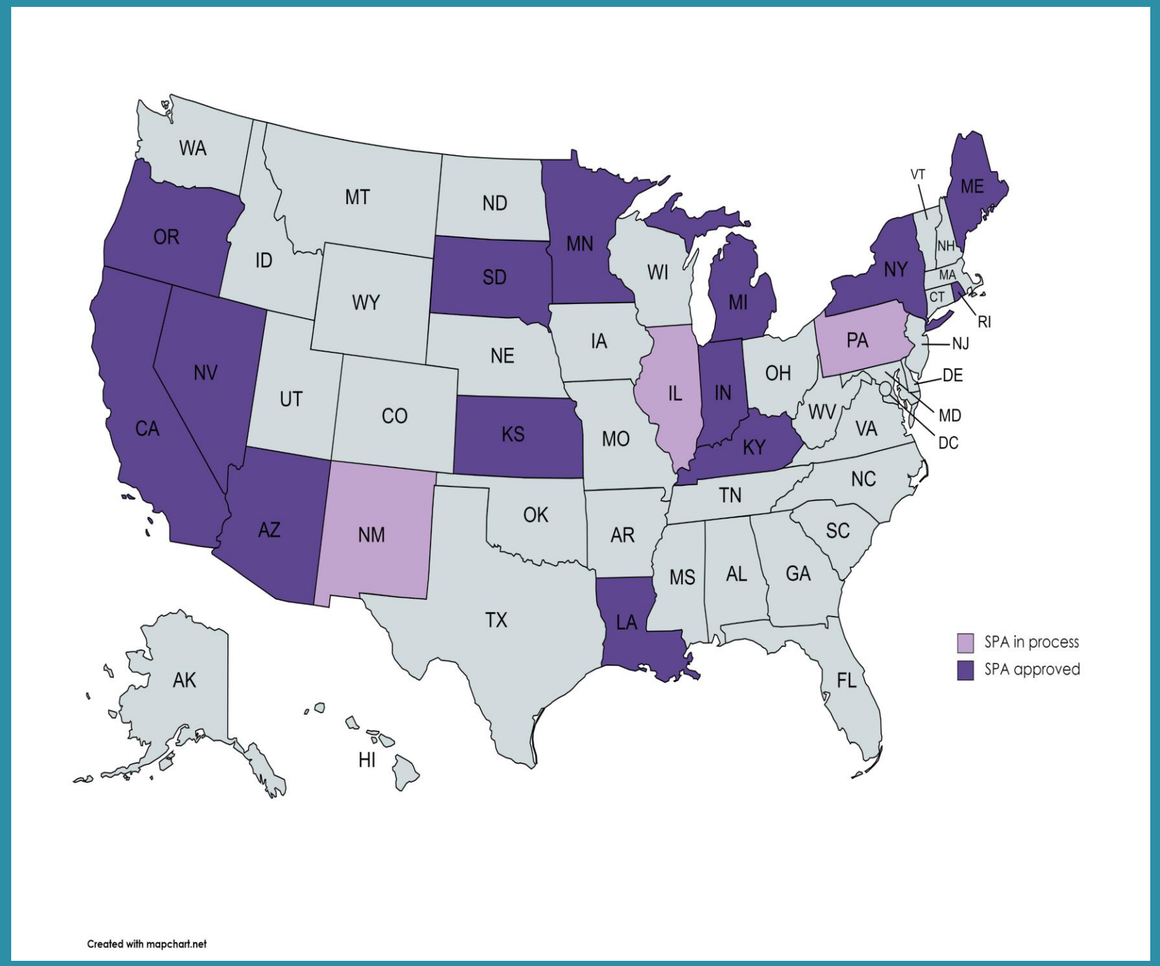
02
How does it impact?

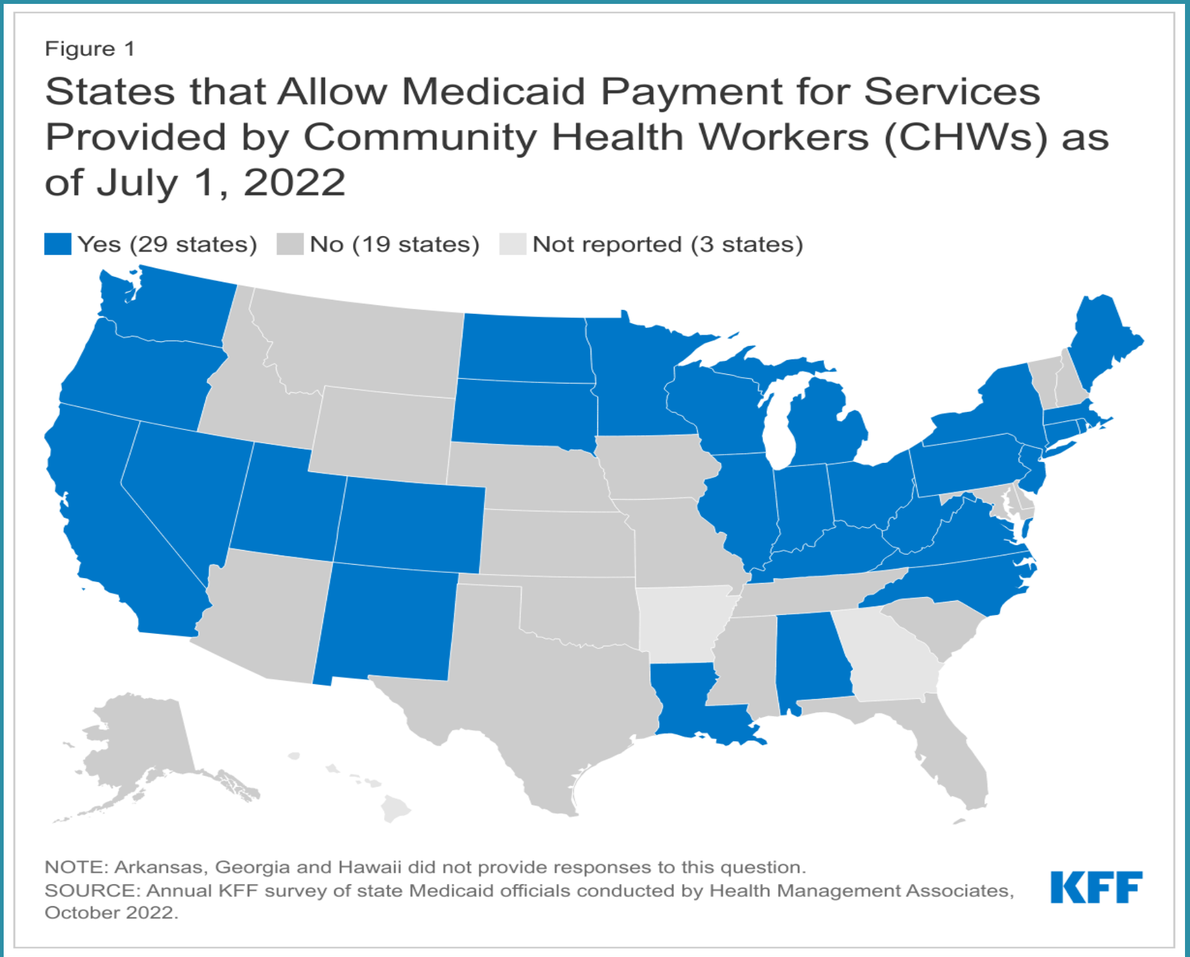
The integration of Community Health Workers (CHWs) or advocates into healthcare systems demonstrates significant benefits across three key areas: reducing disparities, improving outcomes, and lowering costs.

03
How to implement?

Create Advocate-Specific Billing Codes
Tie Reimbursement to Equity Metrics
Exempt regions with advocate shortages (<1 advocate per 1,000 patients) while funding workforce pipelines.

Infographics





States with CHW State Plan Amendments

Financial Barrier: Advocacy programs are often grant-dependent. (workforce & staffing shortages)

Recommendation: Diversifying Funding and Maximizing Impact

01
Why It Matters?

Financial Instability
High staff turnover
Unmet medical assistance
Unmet needs in undeserved communities

02
How does it impact?

Stable salaries
Staff regularly incentivized
Experience Advocates stay
Delivers needed care

03
How to implement?

Private partnerships (Insurers/employers)
Private org paying for advocacy program
Lobby government to dedicate funds for advocacy

Infographic

UnitedHealthcare | Medicare | Individual & family | Medicaid | More UHC sites | Español | Sign in

Shop Insurance | Member resources | Find a doctor | Search

Home > Member resources > Health care programs > Member support and advocacy resources

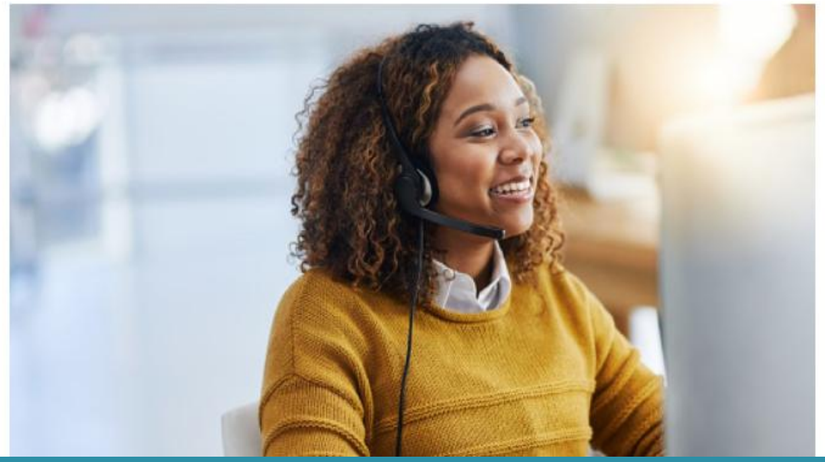
- Health care programs
  - Clinical support programs
  - Employee Assistance Program
  - HouseCalls
  - LGBTQ+ resources
  - Maternity support
  - Member support and advocacy resources
  - Mental health programs
  - Quit For Life

Member support and advocacy resources

At UnitedHealthcare, we think plan members should be able to get straightforward explanations for medical bills, help scheduling appointments and more. That's why we provide advocates at no extra cost to members – connecting you with the resources you need and help you make informed health care decisions.

Advocates offer the support you need, when you need it

When you call an advocate, you'll be connected with a specially trained individual who can help answer questions or find answers for you. Advocates have access to a team of professionals skilled in clinical care, emotional health, pharmacy, special needs, health care costs and medical plan benefits. They're here to provide you with the support you need when you need it, for a more positive and effective experience.



Technological Barrier : Limited access to technical infrastructure

Transform Advocacy with Technology: AI and Telehealth Bridging Healthcare Disparities.

01
Why It Matters?

Gaps in access and communication
Data tracking
Focusing on rural areas
AI flagging high risk

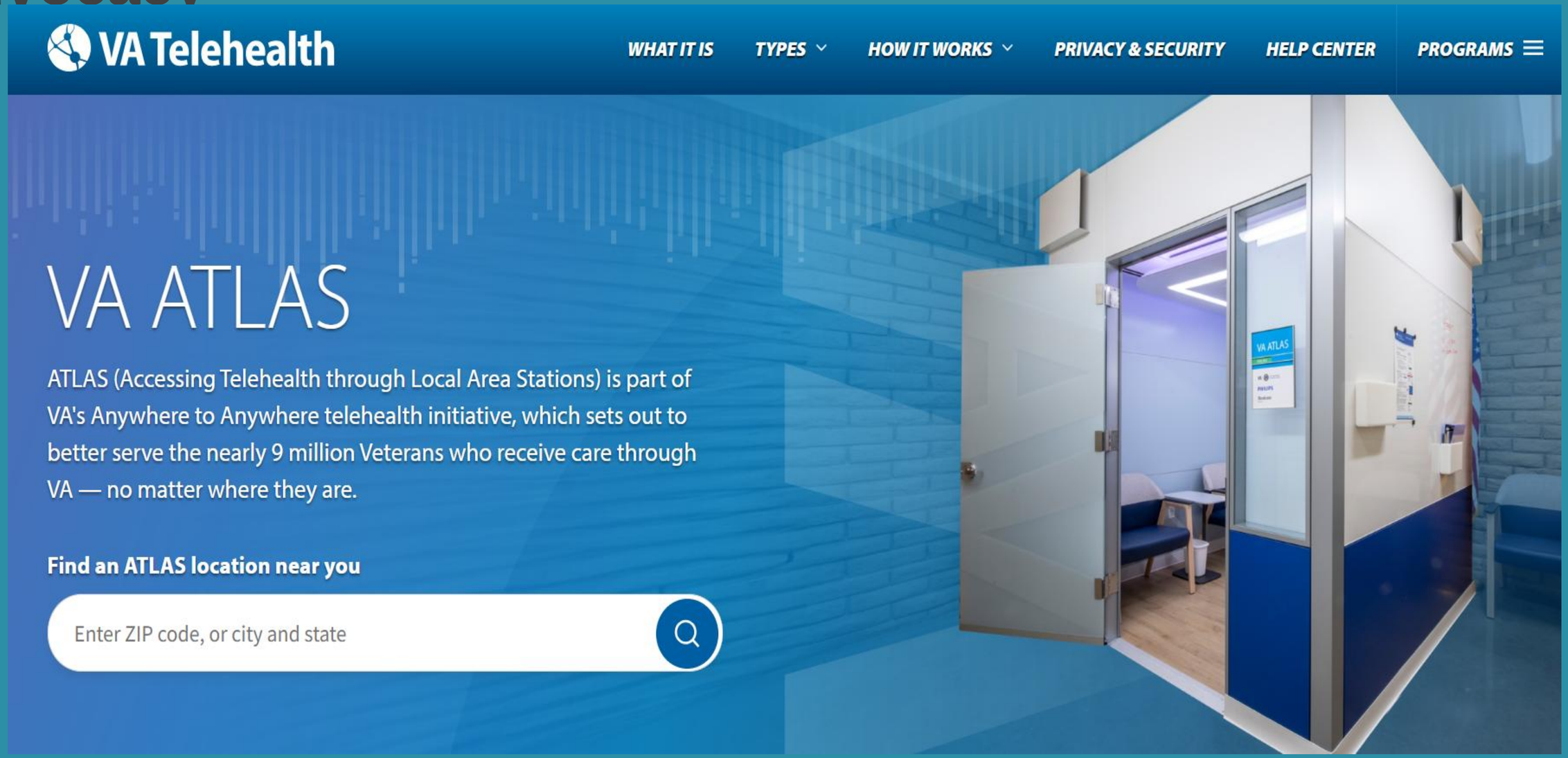
02
How does it impact?

Better access to care
Leverage technological solutions
Integrating advocates into Electronic Health records

03
How to implement?

Partnering with tech firms
Electronic Health records dashboards for advocates
Grants to fund app development for undeserved areas

Leverage Technology for Advocacy



Closing Statement →

Systemic failures—from fraudulent care to exploitative drug pricing—cost lives and trust. Patient advocates prove change is possible: reducing disparities and tackling root inequities. Our roadmap is clear: mandate advocates in care plans, diversify funding, leverage tech, and combat bias. These steps save lives, lower costs, and rebuild trust.

To the AHA and CMS: Imagine a system where advocates have the tools and funding to prevent the next Farid Fata or maternal death. This isn't hypothetical—it's achievable. Let's invest in the infrastructure that makes equity possible.

THANK YOU

References

NIHCM Foundation. (2022, August 2). *The uneven burden of maternal mortality in the U.S.* National Association of Certified Professional Midwives. https://www.nacpm.org/news/the-uneven-burden-of-maternal-mortality-in-the-us-an-infographic-from-nihcm-foundation

Office of Disease Prevention and Health Promotion. (n.d.). *Reduce maternal deaths - MICH-04.*Healthy People 2030, U.S. Department of Health and Human Services.

https://odphp.health.gov/healthypeople/objectives-and-data/browse-objectives/pregnancy-and-childbirth/reduce-maternal-deaths-mich-04/infographic

American Patient Rights Association. (2016, January 18). Revisiting the case of Farid Fata – Why patient advocates must take notice. APHA Blog.

https://aphablog.com/2016/01/18/revisiting-the-case-of-farid-fata-why-patient-advocates-must-take-notice/

Katersky, A. (2024, January 23). Judge upholds 'Pharma Bro' Martin Shkreli's ban from pharmaceutical industry. *ABC News*. https://abcnews.go.com/US/judge-upholds-pharma-bro-martin-shkrelis-ban-pharmaceutical/story?id=106594123

Specter, M. (2021, June 7). How ACT UP changed America. The New Yorker. https://www.newyorker.com/magazine/2021/06/14/how-act-up-changed-america

Black Mamas Matter Alliance. (n.d.). Herstory. Black Mamas Matter Alliance. , from https://blackmamasmatter.org/about/herstory/

Luna, Z., & Luker, K. (2012). Reproductive justice. Annual Review of Law and Social Science, 9, 327-352. https://doi.org/10.1177/2156869312455436

ParityTrack. (n.d.). MHPAEA 10th anniversary., from https://www.paritytrack.org/mhpaea-10th-anniversary/

SupportPROP. (n.d.). Advocacy programs. https://www.supportprop.org/advocacy/

State Policies for Expanding Medicaid Coverage of Community Health Worker (CHW) Services https://www.kff.org/medicaid/issue-brief/state-policies-for-expanding-medicaid-coverage-of-community-health-worker-chw-services/

VA ATLAS https://telehealth.va.gov/atlas

Brazil's Family Health Strategy: Using Community Health Care Workers to Provide Primary Care Brazil's Family Health Strategy: Using Community Health Care Workers to Provide Primary Care | Commonwealth Fund

Community Health Worker Reimbursement MEDICAID AND MEDICARE BILLING IN MINNESOTA Community Health Worker Reimbursement

State Community Health Worker Policies: Texas State Community Health Worker Policies: Texas – NASHP

Cherokee Nation to study ways to improve health access Cherokee Nation to study ways to improve health access | Health | cherokeephoenix.org

Unite Us & North Carolina: A Decade of Enhanced Access to Non-Medical Services Unite Us & North Carolina: A Decade of Enhanced Access to Non-Medical Services

ONE YEAR IN: Massachusetts Hospitals' Historic Health Equity Commitment ONE YEAR IN: Massachusetts Hospitals' Historic Health Equity Commitment - Massachusetts Health & Hospital Association